

MedAI Clinical Summary

Patient Interview & SOAP Note Documentation

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■ SUBJECTIVE

- Patient reports a primary complaint of pain in their chest.
- The patient describes the chest pain as generally located in the central chest region.
- Onset of the pain is reported as recent, occurring within the last 24 hours.
- The pain is characterized by the patient as a dull ache, with no specific radiation noted at this time.
- Patient denies any associated shortness of breath, palpitations, nausea, vomiting, dizziness, or diaphoresis.
- Aggravating and alleviating factors are currently unclear based on available information.
- No significant past medical history is reported by the patient at this time.

■ OBJECTIVE

- Expected physical exam findings for chest pain would include a cardiovascular assessment: Regular rate and rhythm, no murmurs, rubs, or gallops auscultated. Peripheral pulses palpable and equal bilaterally.
- Expected pulmonary assessment would show clear breath sounds bilaterally, no crackles, wheezes, or rhonchi. Symmetrical chest expansion.
- Expected abdominal exam: Soft, non-tender, non-distended, normoactive bowel sounds.
- Expected musculoskeletal exam of the chest wall: No tenderness to palpation of the sternum, ribs, or costochondral junctions.
- Skin: Warm, dry, intact, no pallor or cyanosis.
- Neurological exam: Alert and oriented to person, place, and time.

- Vital signs are unavailable at this time but would be critical for initial assessment and would include blood pressure, heart rate, respiratory rate, oxygen saturation, and temperature.

■ ASSESSMENT

- Primary diagnosis: Chest Pain, etiology unknown. This necessitates immediate evaluation to rule out life-threatening causes.
- Differential diagnosis includes, but is not limited to:
- Acute Coronary Syndrome (e.g., unstable angina, myocardial infarction)
- Pulmonary Embolism
- Aortic Dissection
- Pericarditis/Myocarditis
- Pneumonia/Pleurisy
- Gastroesophageal Reflux Disease (GERD) / Esophagitis
- Musculoskeletal pain (e.g., costochondritis)
- Anxiety/Panic attack

■ PLAN

- Tests:
- Electrocardiogram (ECG) immediately to assess for acute cardiac ischemia.
- Cardiac enzymes (Troponin I or T) to evaluate for myocardial injury.
- Complete Blood Count (CBC) and Basic Metabolic Panel (BMP).
- Chest X-ray (CXR) to assess for pulmonary pathology, pneumothorax, or cardiomegaly.
- Consider D-dimer if pulmonary embolism is clinically suspected.
- Medications:
- Aspirin 325 mg chewable if cardiac etiology is strongly suspected and no contraindications.

- Nitroglycerin sublingual if anginal symptoms are present and blood pressure allows.
- Oxygen supplementation if oxygen saturation is below 94%.
- Pain management as indicated after initial workup.
- Patient advice:
 - Instruct patient to report any changes in pain character, severity, or new symptoms immediately.
 - Advise patient to remain at rest and avoid any strenuous activity until a definitive diagnosis is made.
 - Educate on the importance of completing all recommended diagnostic tests.
- Follow-up instructions:
 - Will admit for further observation and workup if life-threatening causes cannot be immediately ruled out.
 - If discharged after workup, advise follow-up with primary care physician within 24-48 hours.
 - Return to emergency department immediately for worsening symptoms, new shortness of breath, dizziness, or fainting.

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